Improving Public Service Access for High-Risk Suicidal Youth with Abuse Histories in South Korea: Barriers and Facilitators from a Qualitative Study

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Presenter Disclosures

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Background

- In 2025, South Korea’s age-standardized suicide rate was 24.9 per 100,000, 2.3 times the OECD average of 10.9 and the highest among OECD countries.
- Although the overall rate declined from the previous year, the rate among teenagers (aged 10–19) rose by 7.5%, the highest since records began in 1983.
- Suicide is the leading cause of death among Koreans in their teens through forties, and accounts for more than half of all deaths in their teens and twenties.
- Young adults who have experienced abuse or violence are at particularly high risk of suicide.
- However, qualitative research on how they access public services remains limited in South Korea.

Source: Ministry of Data and Statistics, 2025 Causes of Death Statistics (September 2026).

Research Question

What facilitates or hinders public service access, and its continuity, among young adults with abuse histories at high risk of suicide?

Key Findings

- Young adults with histories of abuse and violence encountered multiple public services across the life course, but using a service did not necessarily lead to continuous support.
- **Access went smoothly** where there were easy-to-enter school counseling spaces, free or low-cost counseling, and rapid shelter linkage.
- **Continuity weakened** through guardian involvement and transitions between agencies:
  - Some participants faced blame or pulled away from counseling after guardians learned they had sought help
  - Guardian objection restricted access to treatment
  - Changes in age or residence ended existing support or left young people to search for new services

Methods: Participants & Interviews

1. Participants

- Young adults aged 19–34 (as defined by the Framework Act on Youth, South Korea)
- **Inclusion criteria:** (1) history of abuse or violence victimization; (2) suicidal ideation or suicide attempt within the past year
- Recruited July 3–23, 2025, via 10 online communities and official notices from relevant organizations; 55 applied, and 25 were selected after phone screening for eligibility
- **19 of 25 participants** first engaged in suicidal behavior during elementary, middle, or high school; interviews covered service experiences across the life course

2. In-depth interviews

- Semi-structured; in person by default, Zoom on request (14 in person, 11 via Zoom)
- Mean duration 67.1 min (SD = 16.7); audio-recorded with participants' consent
- **Safety protocol:** interviews were paused or stopped if participants showed or reported distress, with referral to professional support services as needed

Methods: Expert FGIs & Analysis

3. Expert focus group interviews (FGIs)

- 3 FGIs with 13 youth suicide prevention experts, using maximum variation sampling (Patton, 2014)
  - FGI 1: Crisis intervention and postvention (n = 4)
  - FGI 2: Prevention and support across young adults' lives and development (n = 5)
  - FGI 3: Youth policy, education, and youth culture movements (n = 4)
- Recruited via official e-mails and phone calls to relevant institutions (Aug 8 – Sep 3, 2025); small honorarium; recorded with consent

4. Analysis

- **Thematic analysis** (Braun & Clarke, 2006) of interview and FGI transcripts, with peer debriefing among researchers
- Focus: service access, continuity, inter-agency linkage, and care discontinuities
- Approved by the Kyungpook National University IRB (KNU 2025-0366, July 7, 2025); all participants provided informed consent

Results 1. Key Facilitators of Service Access

1. Safe, easily accessible counseling spaces in schools

- **FGI:** School counselors in Wee Class, the in-school counseling office, counseled and supported students affected by abuse or violence

> “The Wee Class teacher treated it like no big deal. It didn’t feel like ‘this kid is sick and needs care’ — it felt like I could just drop in and rest for a bit, so I was more at ease.”

2. Accessible mental health services that are low-cost or free

- **FGI:** Community mental health welfare centers, university counseling centers, suicide prevention centers, and youth counseling and welfare centers provide low-cost or free mental health services to young people who experienced abuse or violence

> “Money was the biggest barrier to going to places like that. I was already struggling emotionally, and about eight counseling sessions would have cost around 50% of my bank balance. (…) Being able to get counseling without worrying about money was the best part.”

Interview quotes were translated from Korean by the authors.

Results 1. Key Facilitators of Service Access

3. Rapid linkage to emergency shelters

- **FGI:** Psychiatric Emergency Response Center links mental health centers, the police, and hospitals to respond to crises such as suicide attempts
- **Interviews:** 3 participants entered a shelter on the same day through a teacher, the police, or a university counselor

"I went to the counseling center and cried for an hour. When they asked if I could tell them what had happened, I only said I'd run away from home. Then they connected me to a short-term youth shelter."

Results 2. Key Barriers to Service Access

1. Heightened risk after parents were notified

- **FGI:** Parental involvement often worsened young people's situations, yet children and adolescents are hard to separate from their parents, which complicates intervention

"I cut my wrist, and the counselor told my parents. My parents said, 'People don't die that easily. If you have that kind of courage, live harder.' (…) They made it look like I just wanted to be some kind of weird person. The counseling itself had helped me, but after what happened it stopped helping. So it was hard to go back to counseling as an adult."

2. The 'service cliff' in the transition to adulthood

- **FGI:** Service delivery is fragmented between child/adolescent and young adult systems, so support is not sustained

"By then I had to pay myself, because I was too old for youth counseling centers. Private counseling was too expensive, so I think I went two or three times and then stopped."

Results 2. Key Barriers to Service Access

3. Fragmented service delivery across institutional and jurisdictional boundaries

- **FGI:** Eligibility and criteria differ by ministry — e.g., Ministry of Gender Equality and Family (adolescents) vs. Ministry of Health and Welfare (young adults) — creating policy blind spots, widened further by differences in program scope across local governments

> “The biggest thing for me was rent support. I went from Daegu to Uljin and back (…), but the referral just didn’t go through. They kept passing me along, so in the end I gave up. If you move, you have to start the whole thing over from the beginning.”

4. Restricted access due to guardian consent requirements

- **FGI:** In practice, guardian consent was treated as required for minors’ inpatient treatment; when guardians objected, practitioners saw no way to proceed

> “I told them I wanted to see a psychiatrist … but they were against it, so I remember not being able to go.”

Implications

1. Safe pathways when guardians block help-seeking

- Let adolescents use services safely; weigh household safety and follow-up protection before and after guardians are notified

2. Clear responsibility for inter-agency linkage

- Name who is accountable for each referral and confirm that young people actually use the next service

3. Transition planning before adolescent services end

- Plan counseling, treatment, and housing support together with adult-serving agencies

Suicidal behavior began in school years for 19 of 25 participants, yet in many cases mental health services connected only after an acute crisis. Sustaining support at each life-course transition opens earlier opportunities for intervention.

Thank you for your attention

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